

Adrabetadex ARM Training Fact Matrix

Working Frame and Master Assumption Source

This matrix is the master assumptions source for the Adrabetadex ARM package. Downstream materials should align to this matrix and avoid duplicating assumption logic unless operational detail is needed.

- **Launch WAC:** \$39,000 per 900 mg vial
- **Dosing:** every two weeks
- **Annual gross WAC exposure:** \$1,014,000 per patient
- **Pre-ASP period:** approximately first two quarters after launch
- **OPPS pass-through effective date:** January 1, 2027
- **Permanent HCPCS effective date:** April 1, 2027
- **Primary customers:** centers of excellence
- **340B status:** assume target COEs are 340B entities
- **Commercial friction:** valid and expected, but mitigated through Beren-provided ARM and patient-access tools
- **Core value story:** scenario-specific economics with a COE-led 340B default and non-340B sensitivity analysis.

Master Assumption Register and Status Labels

Status labels: Confirmed/supplied assumption; Planning assumption; Modeling assumption; External validation needed.

Assumption ID	Assumption	Status Label	Primary Use
A1	Launch WAC is \$39,000 per 900 mg vial with every-two-week dosing and annual gross WAC exposure of \$1,014,000.	Confirmed/supplied assumption	Training baseline and calculator anchor
A2	PDUFA planning date is August 17, 2026; OPPS pass-through target date is January 1, 2027 if granted; permanent HCPCS target date is April 1, 2027 if assigned.	Planning assumption	Timeline planning across all materials
A3	Primary launch customers are COE and pediatric specialty hospitals with a default 340B profile.	Planning assumption	Default scenario selection and ARM talk track
A4	Non-340B cases remain sensitivity scenarios	Modeling assumption	Sensitivity views in calculator and training

A5	and should not be the default value story. Interim coding pathways (for example C9399/J3490/J3590) and modifier use are payer-specific and institution-specific.	External validation needed	Billing reference flags and claim verification prompts
A6	Commercial friction (PA, site-of-care edits, specialty pharmacy sourcing, denial/rework risk) is expected and mitigated through Beren tools.	Confirmed/supplied assumption	ARM execution plan and objection handling

1. Core Product and Launch Facts

Topic	Fact / Assumption	ARM Training Implication	Calculator Treatment
Product	Adrabetadex	Clinician-administered, rare-disease therapy requiring specialty-center readiness	Fixed product
Indication focus	Infantile-onset Niemann-Pick disease type C	Launch volume is concentrated, not broad retail or community-office utilization	Fixed launch population
PDUFA / approval planning date	August 17, 2026	Approval timing drives pass-through and HCPCS sequencing	Fixed date
Target treatment model	Pediatric specialty hospitals and centers of excellence	Training should focus on institutional reimbursement, not broad office-based selling	Default site type = COE / HOPD
Target site economics	Target COEs are 340B entities	Value story should center on 340B acquisition advantage plus outpatient payment visibility	Default 340B = Yes
Clinical value frame	Survival and functional preservation	Supports medical necessity and access case, but does not replace reimbursement execution	Narrative support, not a formula input

The current combined brief already frames the launch as concentrated in pediatric specialty hospitals and centers of excellence, and it states that WAC is \$39,000 per 900 mg vial dosed every two weeks, producing \$1,014,000 in annual gross drug exposure before administration cost or denied claims.

2. Dosing and Cost Exposure Matrix

Item	Value	Calculation	ARM Meaning
WAC per vial / administration	\$39,000	Fixed	Core acquisition exposure per treatment
Dosing frequency	Every 2 weeks	Fixed	26 administrations per year
Annual administrations	26	52 weeks / 2	Recurring high-dollar exposure
Annual WAC exposure	\$1,014,000	$\$39,000 \times 26$	Baseline gross drug exposure
Two-quarter administrations before ASP	13	$26 / 2$	Pre-ASP exposure period
Two-quarter WAC exposure before ASP	\$507,000	$\$39,000 \times 13$	Inventory/payment exposure before ASP benchmark matures
Initial-sales Medicare benchmark	\$40,170	$\$39,000 \times 103\%$	Pre-ASP benchmark reference
Two-quarter benchmark reimbursement	\$522,210	$\$40,170 \times 13$	Reimbursement benchmark before labor, denial, and delay
Two-quarter gross spread before operating costs	\$15,210	$\$522,210 - \$507,000$	Thin gross spread if not using 340B economics

The provider model uses \$39,000 per 900 mg vial every two weeks, \$1,014,000 per patient-year, and a \$40,170 launch-period benchmark based on 103% of WAC before ASP is available.

3. Site-of-Care and Customer Matrix

Site / Customer Type	Operating Fact	Why It Matters	ARM Message
Centers of excellence	Primary target customers	Patients, specialists, pharmacy infrastructure, and rare-disease billing expertise are concentrated here	“Adrabetadex launch will be COE-led.”
Pediatric specialty hospitals	Expected core site type	More likely to manage intrathecal administration, pediatric Medicaid, and rare-disease prior authorization	“The right sites already have much of the clinical infrastructure.”
340B hospitals	Assume all primary COEs are 340B entities	340B acquisition changes provider economics materially	“The site’s 340B status is central to the value story.”
Non-340B community sites	Not the core first-wave target	Prior model showed fragile or negative economics outside 340B	“Non-340B logic is secondary for this ARM training version.”
Physician office	Not primary model	May matter for coding education, but not the core economic pathway	“Keep as background, not the main value story.”

The existing model shows 340B pediatric COE economics as structurally advantaged, with acquisition of \$29,991, modeled margin of \$9,043 per administration, and annualized margin of \$235,115, while non-340B commercial and Medicaid cases are near break-even or negative.

4. 340B Economics Matrix

340B Item	Fact / Assumption	Value	ARM Implication	Calculator Treatment
Target site 340B status	COEs are assumed to be 340B entities	Yes	Default customer economics should be modeled as 340B, not as non-340B	Default = 340B
WAC	Launch WAC	\$39,000	Acquisition reference point	Fixed
Estimated 340B acquisition, primary assumption	WAC less 23.1% estimate	\$29,991	Creates major spread advantage	Default 340B acquisition

340B Item	Fact / Assumption	Value	ARM Implication	Calculator Treatment
Estimated 340B acquisition, sensitivity	WAC less 17.1% estimate	\$32,331	Shows sensitivity if pediatric rebate category differs	Sensitivity toggle
Acquisition advantage vs WAC, 23.1% case	\$39,000 - \$29,991	\$9,009 per dose	Explains why 340B is the main site value lever	Display prominently
Annual acquisition advantage, 23.1% case	$\$9,009 \times 26$	\$234,234 per year	Converts 340B into an annual site-level economics story	Display as annualized value
Two-quarter acquisition advantage, 23.1% case	$\$9,009 \times 13$	\$117,117	Shows pre-ASP / early launch value	Display for launch bridge

The combined brief identifies estimated 340B acquisition at \$29,991 under the 23.1% URA assumption and \$32,331 under the 17.1% URA assumption, and it states that 340B participation fundamentally changes the provider result.

5. Coding and Payment Timing Matrix

Workstream	Operating Fact	Date / Timing	What ARMs Need to Know	Training Message
FDA approval	Approval gates commercial billing and pass-through package completion	August 17, 2026	Approval letter and final label are needed for CMS processes	"Approval starts the reimbursement clock."
Launch coding	Product-specific HCPCS not active at approval	August 2026 launch window	Interim billing is required	"Early claims need clean interim coding support."
Interim hospital outpatient coding	C9399 or NOC pathway may be used depending on setting and payer	Approval to permanent-code effective date	Requires NDC, dosage, narrative, and clean documentation	"Billing guide must be ready before first commercial dose."

Workstream	Operating Fact	Date / Timing	What ARMs Need to Know	Training Message
OPPS pass-through	Separate from permanent HCPCS	Effective January 1, 2027	Provides near-term HOPD payment support and predictability	“Pass-through is the outpatient payment bridge.”
Permanent HCPCS	Product-specific permanent code	Effective April 1, 2027	Improves claim clarity and reduces miscellaneous-code friction	“Permanent HCPCS is the clean-code endpoint.”
ASP availability	Not available at launch	After sufficient sales/reporting lag	Pre-ASP payment uses WAC-based benchmark	“Before ASP, WAC is the benchmark anchor.”

The coding brief states that with a supplied August 17, 2026 PDUFA date, the first full quarterly coding window after approval is October 1, 2026, targeting an April 1, 2027 effective date for a permanent product-specific HCPCS code. It also states that a September 1, 2026 pass-through filing can target a January 1, 2027 effective date.

6. Launch Period Timeline Matrix

Period	Dates	Coding / Payment State	ARM Training Focus	Calculator Mode
Launch to year-end	Aug. 17, 2026 to Dec. 31, 2026	FDA-approved, no permanent HCPCS, pre-pass-through, pre-ASP	Interim billing, documentation, NDC capture, prior authorization tools	Launch bridge mode
Pass-through active, pre-HCPCS	Jan. 1, 2027 to Mar. 31, 2027	OPPS pass-through active, permanent HCPCS not yet active	HOPD payment predictability improves, but interim coding still matters	Pass-through bridge mode
Permanent HCPCS active	April 1, 2027 onward	Product-specific HCPCS active	Cleaner claims, reduced miscellaneous-code burden, stronger provider confidence	Permanent-code mode
ASP maturation	After first two quarters of	ASP-based payment begins once available in CMS files	Update assumptions once ASP is published	ASP update mode

Period	Dates	Coding / Payment State	ARM Training Focus	Calculator Mode
	commercial data/reporting			

7. Commercial Friction Matrix

Friction Point	Operating Fact	Beren Mitigation Tool	ARM Message	Calculator Relevance
Prior authorization	Valid and expected	Pre-authorization letter / medical necessity template	"We expect PA, and the team is equipped to support it."	Denial probability / approval status
Genetic confirmation	Expected in analogous NPC policies	Documentation checklist	"Genetic confirmation should be prepared before submission."	Documentation completeness
Specialist prescribing / oversight	Expected	Prescriber attestation / clinical support packet	"Specialist involvement should be positioned as expected, not exceptional."	PA readiness
Continuation criteria	Expected	Renewal / continuation support template	"Reauthorization must be planned from baseline."	Reauth timing / continuation risk
Site-of-care review	Valid and expected	Site-of-care support rationale	"COE administration should be framed as clinically and operationally appropriate."	Site approval risk
White-bagging / specialty pharmacy sourcing	Valid risk	Channel-specific payer response pathway	"White-bagging may reduce inventory exposure but can remove drug spread."	Buy-and-bill vs white-bagging toggle
Denials	Expected in launch gap	Denial letter / appeal template	"Denials are manageable only if the response pathway is ready."	Denial/rework cost
NDC / claim documentation	Required for clean payment	Billing guide	"Clean claim construction is part of the access strategy."	Claim quality input

The combined brief documents that analogous commercial policies use genetic confirmation, specialist oversight, continuation based on documented positive response, overlap controls, precertification, site-of-care management, and administration requirements. It also identifies specialty-pharmacy sourcing as a separate access-control channel that can determine whether the provider bills for the drug or only administration.

8. ARM and Patient-Team Tool Matrix

Tool	Primary User	Purpose	Where It Fits in Workflow	Training Message
Pre-Authorization Letter	ARM / patient access team / provider office	Supports initial payer approval	Before first dose	"Do not let the site build the PA record from scratch."
Medical Necessity Letter	Provider / patient team	Establishes clinical rationale	PA and appeal packet	"Tie diagnosis, disease severity, treatment rationale, and site-of-care need together."
Denial Letter / Appeal Template	Patient team / provider billing	Responds to payer denial	Post-denial / resubmission	"Denial response should be standardized and fast."
Billing Guide	ARM / provider billing	Supports HCPCS/NOC/C9399 /NDC claim construction	Before first commercial claim	"Billing readiness should precede dosing."
Site-of-Care Rationale	ARM / provider / payer access team	Defends COE or hospital outpatient administration	PA and site-of-care review	"COE care is part of the clinical and operational logic."
Reauthorization Checklist	Patient access team / provider	Supports continuation coverage	Before renewal date	"Continuation evidence starts with baseline documentation."
Net-Cost Recovery Calculator	ARM	Shows provider economics	Site value discussion	"This converts reimbursement mechanics into site-level economics."
One-Page PDF Output	ARM / provider stakeholder	Summarizes scenario-specific value	Leave-behind	"Use as the executive summary of the calculator result."

9. Medicaid / EPSDT / PAD Matrix

Topic	Fact / Assumption	ARM Training Point	Calculator Treatment
Medicaid relevance	Meaningful due to pediatric population	Medicaid should be treated as a core payer segment	Separate Medicaid scenario
EPSDT	Coverage tailwind for medically necessary pediatric care	Helps the coverage argument, not the payment formula	Narrative support
PAD	Adrabetadex is treated as physician-administered / clinician-administered drug logic	NDC and claim integrity matter	Claim quality field
Medicaid managed care	May add payer-specific PA and sourcing controls	Do not assume FFS and MCO are the same	FFS vs MCO toggle
340B Medicaid handling	Duplicate-discount controls matter	Site must manage carve-in / carve-out accurately	340B Medicaid flag
COE assumption	COEs are 340B entities	Medicaid economics should not default to non-340B loss model for target sites	Default Medicaid COE = 340B

The prior provider model correctly shows that Medicaid non-340B economics are negative, but under the revised working facts, the ARM-facing model should default target COE Medicaid scenarios to 340B because the primary customers are 340B entities. The concept brief still remains useful for showing why 340B status changes behavior and why Medicaid claim integrity cannot be ignored.

10. Value Story Matrix

Value Story Element	Current Fact	ARM Translation	Proof / Model Output
Clinical need	Ultra-rare, severe pediatric disease	COEs understand the clinical need, but need reimbursement certainty	Clinical narrative
High acquisition exposure	\$39,000 per dose, \$1.014M annually	Sites need confidence before stocking and billing	Acquisition exposure output

Value Story Element	Current Fact	ARM Translation	Proof / Model Output
Target-site concentration	COEs are the main customers	One core training story can apply across most target accounts	Site profile field
340B advantage	Target COEs are 340B entities	Main economic lever is acquisition advantage vs WAC	Per-dose and annual 340B value
Pass-through	Effective Jan. 1, 2027	HOPD payment predictability improves early in launch	Pass-through scenario
Permanent HCPCS	Effective Apr. 1, 2027	Claim clarity improves after the bridge period	Permanent-code scenario
Beren support tools	PA, denial, billing, and access tools	Commercial friction is expected but addressable	Tool checklist
Provider economics	340B COE margin is structurally advantaged	"The economics can work at the right sites."	Net margin output

11. Revised Calculator Default Assumption Matrix

Input	Old / Broad Model	Revised ARM Training Default
Site type	HOPD / office / community / COE	COE / pediatric specialty hospital
340B	Toggle	Default = Yes
WAC	\$39,000	\$39,000
Dosing	Every 2 weeks	Every 2 weeks
Annual administrations	26	26
Pre-ASP administrations	13	13
Pass-through	Scenario-dependent	Effective Jan. 1, 2027
Permanent HCPCS	Scenario-dependent	Effective Apr. 1, 2027
Commercial friction	Risk factors	Expected, mitigated with Beren tools

Input	Old / Broad Model	Revised ARM Training Default
Non-340B support	Major scenario	Secondary / sensitivity only
Core output	Compare all channels	Show 340B COE value and operational readiness

12. Final ARM Training Thesis

Adrabetadex launch adoption will be COE-led. Because the target centers are 340B entities, the provider value story is not primarily about rescuing fragile non-340B economics. It is about showing 340B centers that Adrabetadex can be administered through a predictable reimbursement pathway once pass-through, interim billing support, permanent HCPCS timing, and payer-access tools are understood.

The practical message:

At the target sites, the economics can work. The ARM job is to make the pathway visible, reduce reimbursement uncertainty, and show the institution how 340B acquisition, pass-through timing, clean billing, and Beren's access tools convert clinical interest into operational adoption.

Codex implementation mapping

Assumption ID	Training Guide	PowerPoint	Calculator	One-Page PDF	Billing Reference	Notes
A1	Executive summary figures and key assumptions section	Opening quantitative frame slide	Default WAC/dose/frequency inputs	Top-line numbers block	Referenced as planning baseline only	Scenario-specific economics; does not guarantee payment
A2	Launch timeline and sequencing language	Timeline slide with if-granted/if-assigned qualifiers	Date-state toggles for launch/pass-through/HCPCS	Timeline callout row	Timeline table with verification prompts	Mark as planning timeline pending external confirmation
A3	COE-led default scenario positioning	Target-account prioritization slide	Default site type = COE/HOPD and 340B = Yes	Default scenario summary	Hospital outpatient checklist emphasis	Institution-specific validation still required

A4	Non-340B described as sensitivity only	Sensitivity appendix slide	Sensitivity toggle only (non-default)	Sensitivity footnote	Professional/commercial pathway caution	Avoid presenting non-340B as expected baseline
A5	Reference's billing reference for code-level details	Compliance and verification slide	No hard-coded billing guarantee outputs	Validation note only	Primary home for procedural coding flags	External validation needed before claim submission
A6	Objection handling and ARM workflow section	Access-friction mitigation slide	Denial/rework and sourcing assumptions	Risk disclaimer row	Checklist and reminder blocks	Use cautious language: expected/verify/payer-specific